

HEALTH CLUSTER BULLETIN - MOZAMBIQUE

Humanitarian Context

Mozambique continues to face a complex humanitarian crisis characterized by the overlapping impacts of climate-related shocks and armed conflict, significantly affecting the health and well-being of vulnerable populations. In 2026, floods in the central and southern provinces resulted in population displacement, damage to health infrastructure, and an increased risk of waterborne disease outbreaks, disrupting the continuity of essential health services.

At the same time, insecurity in Cabo Delgado continues to drive displacement and place additional strain on already fragile health systems. Humanitarian needs remain high, with priority given to the provision of primary health care, emergency services, mental health support, and assistance to displaced populations and host communities.

In this context, Health Cluster partners have maintained a coordinated response to ensure the delivery of essential health services, with a focus on integrated approaches including mobile clinics, outbreak response, sexual and reproductive health services, and water, sanitation, and hygiene (WASH) interventions. However, significant challenges persist, particularly in terms of access, resource availability, and the need to strengthen monitoring and reporting mechanisms.

This bulletin provides an overview of the activities and achievements of Health Cluster partners during April 2026, highlighting collective efforts to respond to urgent needs and support the recovery of affected communities in the northern region (conflict) and southern region (floods).

1. KEY FIGURES FOR THE HRP 2026 NORTHERN AREA (CABO DELGADO, NAMPULA AND NIASSA)

Under the 2026 Humanitarian Response Plan, the Health Cluster will address three strategic objectives, monitored through 15 key indicators. These activities aim to reach 409,968 beneficiaries by the end of the year in 18 districts: 15 districts in Cabo Delgado, 2 districts in Nampula province (Memba and Erati) and 1 district in Niassa province (Mecula).



1.1 Indicators reported in April 2026

HRP 2026 Indicators	HRP Target 2026	Jan	Feb	Mar	April	% reached of target
Number of health care workers trained	150	0	41	166	22	138.00%
Number of disability related consultations	34354	0	825	0	845	2.40%
# of total OPD consultations	378538	40304	13218	64417	8651	31.16%
# of normal deliveries attended by skilled birth attendants	378538	13705	378	0	0	3.72%
# of community members receiving health IEC messages	378538	1237	0	1750	2622	0.79%
# of cases receiving mental health and psychosocial support services	34354	nr	nr	nr	311	0.91%

Reported indicators Health Cluster 5W matrix April 2026.

However, gaps in reporting persist for certain indicators, attributed in some instances to partner reporting challenges, and in others to a lack of adequate monitoring and reporting tools. The Health Cluster is actively working with partners to address these gaps and strengthen overall reporting and monitoring capacities.

2. Health Cluster partners activities carried out in the Northern (conflict response) during the month (April_2026)

Partner	Activity
Action Contre la Faim (ACF)  <i>Figure 5: Lecture on Malaria Prevention and the Importance of Vaccinating Children Under 2 Years of Age in Namange Community - Quissanga District, April 2026</i>  <i>Figure 6: Patient care at the Sick Child Consultation (CCD) - Imbada Community, Meluco District, April 2026</i>	During the current month ACF, carried out: Mobile brigade's and clinics activities: - Held in all three districts: Macomia, Meluco, and Quissanga. Mobile brigades were conducted in twenty (20) communities, specifically in Macomia (Songueia Centro, Ntapuala, Bangala-1, and Licangano) Meluco (Pedreira, Massasse, Mitambo, Nguia, Mariria, Pitonha, Citate, and Nsemuco), and Quissanga (Quissanga 2 and Milamba), - Mobile Clinics team carried out activities in Macomia, Quissanga and Meluco districts. In Macomia the mobile clinic was held in the communities of Bangala 2, Nanao resettlement. In Meluco district, the Mobile Clinics took place in the communities of Nangororo, Imbada and Ncoripo. In Quissanga, the mobile clinic took place in 1° de Maio, Namange and Indique. Referral of critically ill patients from the community to provincial hospital in Pemba: - In the district of Meluco, 7 cases of critically ill patients were referred from the communities of Namagico (1), Nsemuco (2), Citate (2), and Mitambo (2) during the month of April. In Macomia, 10 patients were referred, and in the district of Quissanga, 8 critically ill patients were referred to the Provincial Hospital in Pemba.
International Organization for Migration (IOM) 	During the current month IOM, carried out: - Integrated Mobile Brigades in Chiure, Metuge, Memba, and Erati reaching host communities and resettlement sites providing a total of 8,226 health consultations. - Mental Health consultations were provided in Memba and Erati reaching 225 people. - Sexual Reproductive Health reaching 1901 (Female 1109 and Male 792) young vulnerable people 1266, migrants 225 and sex workers 1890 reached in Montepuez, 576 (Female 329 and Male 247) young vulnerable people 474, migrants 39 and sex workers 64, referred for health facilities in Montepuez, through sexual and reproductive health programming.

World Health Organization (WHO)



Figure 8: Visit to the Cholera Treatment Centre (CTC) in the Ilha de Moçambique District

During the current month WHO, carried out:

- Supported the National Institute for Health (INS) in implementing activities to investigate cholera-related rumors in communities in the districts of Monapo, Nacala-Porto, and Mogovolas in Nampula Province by providing direct funding for vehicle logistics, meal allowances for community leaders, and stipends for technical staff.
- Provided technical support to the district CTCs (Monapo, Nacala-Velha, and Mogovolas)
- Reviewed and supported the workflow within the CTC in Ilha de Mocambique district;
- Assigned and provided on-the-job training to technicians on how to fill out individual patient forms;
- Assisted in assessing needs and discussing solutions with provincial authorities; and preparing the chlorine solution.

UNICEF



Figure 9: Baivo/UNICEF/Erati District/ IMB in Mulovola Community

During the current month UNICEF, carried out:

- As response to internal displacements and host communities' emerging health care needs, UNICEF deployed 58 integrated mobile brigades (IMBs) to provide life-saving primary health care services in the districts of Erati and Memba.
- These brigades reached a total of 6,580 beneficiaries, which included 3,387 children, of whom 1,852 were girls.
- Furthermore, the services reached 2,653 women, including 476 pregnant women who received antenatal care services.

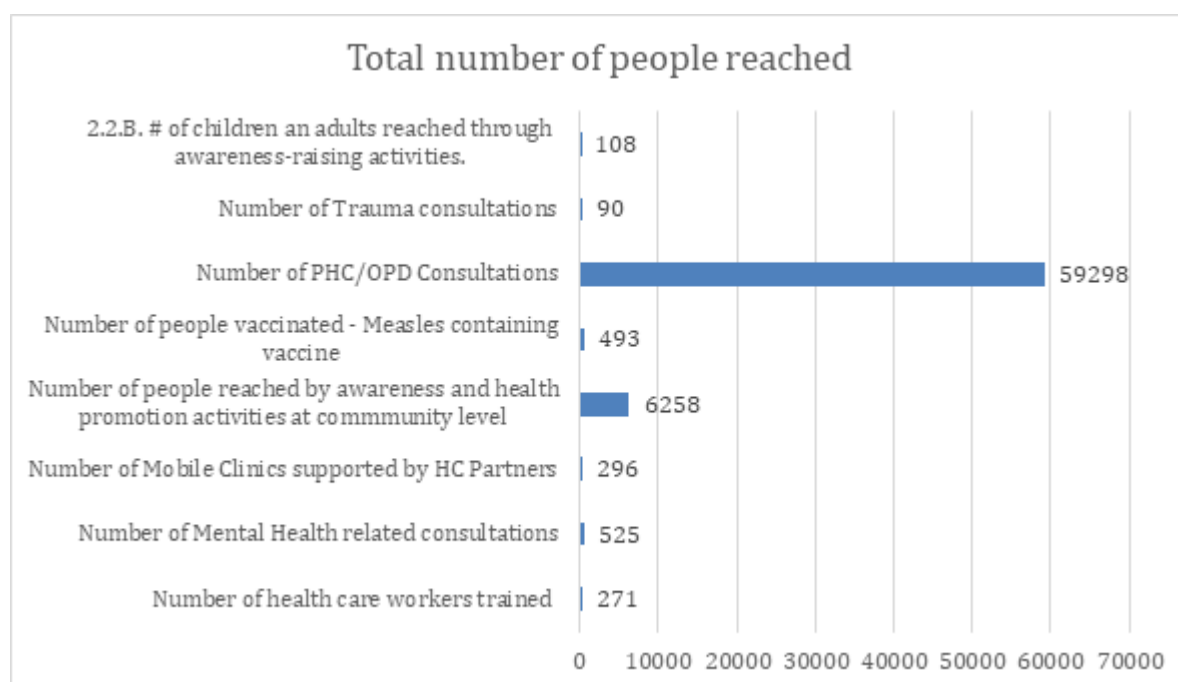
FLOODS RESPONSE IN GAZA, SOFALA, MAPUTO AND MAPUTO CITY

2. KEY FIGURES FOR ADDENDUM PLAN FOR FLOODS 2026

For the 2026 humanitarian response plan addendum for floods, the health cluster will respond to three objectives and from these objectives, 15 indicators will be monitored reflecting activities that will cover by the end of the year a total of 538,725 beneficiaries from the total of people in need (619,627) in 18 districts in Gaza, Sofala, Maputo and Maputo City. In total more than 67,000 people have reached, of which 40,000 in Gaza.



2.1 Indicators reported in April, 2026



Health Cluster 5W matrix April 2026.

In case no partner has reported on an indicator that indicator is not shown in the matrix.

There is a lack of reporting for some indicators, some due to reporting issues from the partners, and some others lack adequate tools to report and monitor. As Health Cluster we are working with partners to improve reporting and monitoring.

3. Health Cluster partners activities carried out during the month in response to the floods (April_2026)

Partner	Activity
UNFPA  	In April, UNFPA supported emergency response across five flood-affected districts in Gaza: Xai-Xai, Guijá, Chókwè, Mabalane, and Mapai. Mobile brigades provide integrated sexual and reproductive health services to over 12,000 people, including family planning and antenatal care. Twelve SRH kits – enough to serve approximately 150,000 people – were distributed, containing essential medical-surgical supplies and medicines for emergency-affected populations. In resettlement sites, UNFPA established three safe spaces: two for women and girls, and one for boys and young men. These spaces offer a safe and welcoming environment for sharing experiences, building skills, and accessing SRH information. Additionally, 1,000 dignity kits with hygiene and personal safety items were distributed to women and girls, supporting their protection, reducing exposure to gender-based violence, and improving access to essential services. These actions have significantly strengthened access to sexual and reproductive health services and GBV prevention and response within Gaza's humanitarian response.

This bulletin is produced by the Mozambique Health Cluster. We acknowledge the support of all government agencies and health partners for their role in providing life-saving primary health services in the affected locations. Equally, we appreciate all the contributions of reports and information from field experiences while delivering the humanitarian response.

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Useful links

- ReliefWeb: <https://mozambique.healthcluster.org>